Message Report

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Generated: 12/19/2025 at 06:19 PM by Nuha Sayegh

Timezone: America/Los_Angeles

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Parents: Fahed Sayegh, Nuha Sayegh

Child(ren): Jordan Sayegh, Mia Sayegh

Date Range: 12/15/2025 — 12/15/2025

Contains: 1 selected messages

Order: Chronological (oldest first, most recent last)

Message 1 of 1

Sent: 12/15/2025 at 09:31 PM
From: Fahed Sayegh
To: Nuha Sayegh *(First Viewed: 12/16/2025 at 09:16 AM)*
Subject: Re: Children's Hospital

Ok,

Let's do this!

> Sent: 12/15/2025 at 09:30 PM
> From: Nuha Sayegh
> To: Fahed Sayegh *(First Viewed: 12/15/2025 at 09:30 PM)*
> Subject: Re: Children's Hospital
>
> The child needs to wear glasses consistently and use the atropine drops as prescribed to prevent permanent vision loss in the right eye.
>
> PERIOD

> Sent: 12/15/2025 at 09:26 PM
> From: Fahed Sayegh
> To: Nuha Sayegh *(First Viewed: 12/15/2025 at 09:27 PM)*
> Subject: Re: Children's Hospital

What the doctor found (key diagnoses)

1) Congenital ptosis (droopy eyelid), right eye — status post repair 7/17/24

- They note an excellent result and that the eyelids look symmetric today.
- Translation: the eyelid surgery looks good right now, and they're not recommending another operation.

2) Refractive amblyopia, right eye (OD)

- "Amblyopia" = the brain has learned to rely less on the weaker eye, so vision development is behind.
- "Refractive" = the cause is mainly the glasses prescription / focusing difference, not a structural eye problem.
- They specifically note poor compliance with glasses and patching, which is a big deal: amblyopia treatment works best when done consistently.

3) Exophoria

- This is a tendency for the eyes to drift outward (not necessarily a constant "lazy eye").
- They say it's small and benign, with no nerve palsies (reassuring).
- "Stereo reduced" = depth perception is decreased right now (common with amblyopia and alignment issues).

4) Anisometropic astigmatism (OD >>> OS)

- "Anisometropic" = the two eyes have very different prescriptions.
- "Astigmatism" = blur from an irregular focus/shape.
- OD>>>OS means the right eye's astigmatism is much higher than the left, and it's grossly stable (not changing dramatically).

5) DFE WNL OU

- Dilated eye exam was within normal limits in both eyes.
- Translation: the inside/back of both eyes looked healthy.

The prescription (Final Rx)

- Right eye (OD): -4.00 +5.00 x 103
- That is a strong prescription with very high astigmatism.
- Left eye (OS): -1.00 +1.25 x 80
- Mild-to-moderate correction.

Why this matters: that big difference between eyes is a classic setup for amblyopia—the brain tends to "pick" the clearer eye unless you force the weaker eye to work (glasses + patching or atropine/other therapy).

The plan (what they want you to do)

A) Glasses: "Released SRx for full time wear."

- This is the foundation. Full-time wear is usually the #1 non-negotiable step for refractive amblyopia.

B) If patching isn't happening, they're offering alternatives

They wrote:

- Atropine … OS 2 days a week … instead of patching
- And they mention VR Luminopia (a digital amblyopia therapy option).
- Translation: If patching is a battle, they may use atropine drops in the left eye (the stronger eye) to blur it and make the right eye work more.

Important: The dosing line is a little confusing as written ("BID…2 days a week…AM"). Before you start, I'd confirm the exact atropine schedule with the clinic/pharmacy instructions.

Follow-up

- Return in 4 months (or sooner PRN = as needed).

That's consistent with amblyopia management—do treatment, reassess progress, and adjust.

What you should focus on at home (high-impact)

1. Glasses every day, all day (comfort + fit matters: straps, proper nose bridge, kid-proof frames).
2. Pick one amblyopia strategy and stick to it consistently:

- Patching (if doable), or
- Atropine penalization, or
- Luminopia (if you go that route)

3. Track adherence like it's medicine (simple calendar checkmarks). The doctor already flagged compliance as the obstacle.

Questions to ask CHLA (so it's crystal clear)

- "Can you confirm the atropine instructions exactly (how many drops, which days, morning vs twice/day)?"
- "If we do atropine, do we still patch at all—or no?"
- "How many hours/day of patching would you want if we recommit to patching?"
- "What is the goal for the next 4 months—improve acuity in OD by how much?"
- "Any concerns about the exophoria becoming a constant outward drift?"

Quick safety note on atropine (if that's the plan)

Common: light sensitivity and blurred near vision in the treated eye. Rare but important: facial flushing, fever, rapid heartbeat, severe irritability—if you see concerning symptoms, call the clinic.

Sent: 12/15/2025 at 09:22 PM

From: Nuha Sayegh

To: Fahed Sayegh *(First Viewed: 12/15/2025 at 09:22 PM)*

Subject: Children's Hospital

I took Jordan to his every four months Ophthalmology appointment and his vision is not getting any better. I would not leave without a solution so we're going to try a different plan. He has been extremely busy the doctor and unable to work on his personal plans due to a public

hospital. great doctors, but too many patients.

I am looking for another doctor because from today's appointment his vision got worst.
HE MUST WEAR HIS NEW GLASSES CONSTANTLY Doc said along with eye patch for an hour each day AND VR glasses each day for an hour. This is not a joke we must be proactive in this.

See Attachments: IMG_0853.jpg (4.9 MB)